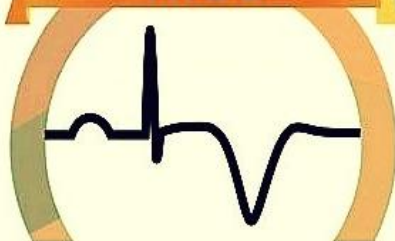


WELLENS TYPE B



Deeply-inverted anterior T waves, not always yet accompanied by chest pain

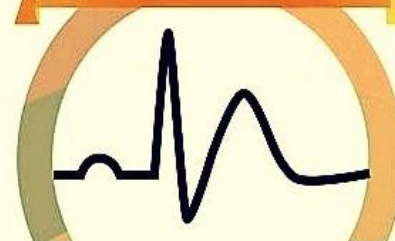
11

UNFORGETTABLE ECG PATTERNS STRONGLY SUGGESTIVE OF CORONARY OCCLUSION

Dr. Antoine Ayer



SHARK T



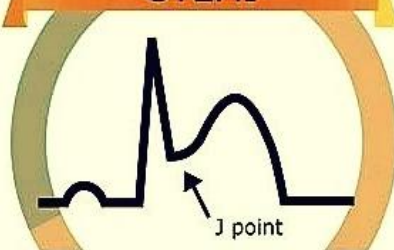
J-point depression transitioning in a convex ST segment

WELLENS TYPE A



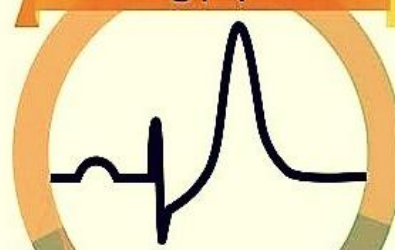
Biphasic anterior T waves, not always yet accompanied by chest pain

CONVENTIONAL STEMI



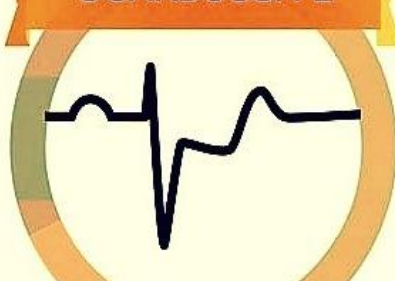
Classical elevation of ST segment measured at (or 40-60ms after) the J point

DE WINTER ST-T



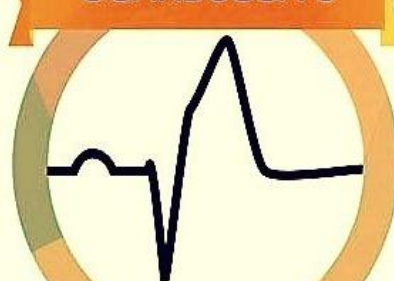
Upsloping J point ST depression in V1-V6 that continues into tall, positive symmetrical T waves, often combined with a 1-2mm elevation of the ST-segment in aVR

SGARBOSSA 2



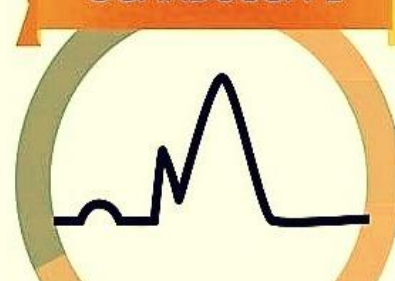
ST depression ≥ 0.1 mV in the same direction as the QRS in V1 to V3, where the LBBB or paced QRS complex is negative

SGARBOSSA 3



ST elevation ≥ 0.5 mV in the opposite direction as the LBBB or paced QRS

SGARBOSSA 1



ST elevation ≥ 0.1 mV in the same direction as the QRS, in the leads where the LBBB or paced QRS complex is positive

POSTERIOR STEMI



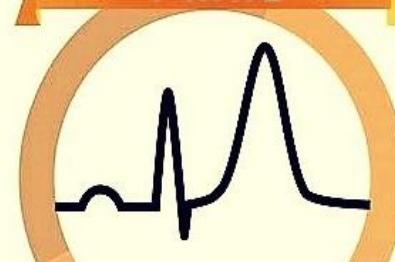
V1-V3(V4) ST depression ≥ 0.05 mV (possibly horizontal or downsloping and concave) especially if there is a concurrent tall R wave in V1/V2 with an R/S ratio > 1 in V2.

LEFT VENTRICULAR HYPERTROPHY



ST elevation $> 25\%$ of QRS amplitude AND [presence of STE in 3 contiguous leads OR presence of T-wave inversions in the anterior leads]

HYPERACUTE T WAVE



Tall, often asymmetrical, broad-based anterior T waves often associated with reciprocal ST depression